



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : وجيده دياب محمد عبد الرحمن

المحافظة : القاهرة الحي / المدينة : الزاوية الحمراء العنوان : ----

رقم البطاقة : 27306240103145 رقم التليفون : 01150712733 تاريخ تسجيل الإستمارة : 29-09-2026

العمر (تاريخ الميلاد) : 24-06-1973 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه (-----) التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : ----- التوقيع : -----

الاسم : د/ندى حسن الوظيفة : ----- التوقيع : -----

الاسم : د/نرمين رمضان الوظيفة : ----- التوقيع : -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /أ.د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي : -----

خطة العلاج : -----

المستشفى : ----- قسم : -----

جهة العلاج : ----- نفقات : -----

ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: وجيده دياب محمد عبد الرحمن National ID: 27306240103145	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية العراق
وزارة الداخلية
مديرية الأحوال المدنية

الجنس

أبي محمد عبد الرحمن

٢٤ من البرزخ

الفرص - الكوفة



٢٧٣.٠٦٢٤.٠١.٣١٤٥

KU4887030



٢٠٢٤/١٠

٢٧٣.٠٦٢٤.٠١.٣١٤٥

(بداية سريان)



مكتوبة

مستند

الحمد سيد عبد الجبار الصافي

الطبعة سارية حتى ٢٠٣١/٠٩/٢٠

١١



Patient Name : الأمثلة / وجيده دياب محمد عبدالرحمن
 Patient No. : 27306240103145
 Path No. : SR-1361/23
 Sex : Female
 Age : 50 Y
 Referring Doc :

Request No. : 13001729
 Request Date : 09/11/2023
 Verification Date : 26/11/2023
 Financial Category : فئة الدولة
 Department : Out Patient
 Registered by : marwa mohamed

Pathology Report

Clinical Data: Right breast cancer and Left axillary lymph node.
 Nature of Specimen: Slide Revision

Gross Examination:

2 paraffin block(s) labeled by (7914/23) and (3) slides were received.

Microscopic Examination of Right breast:

Histological type: Invasive duct carcinoma.

Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade III (3+3+2)

DCIS: No.

Angioinvasion: No.

Perineural invasion: No.

PT:

ER: Strong nuclear reaction in 10% of tumor cells (Allred score 5/8).

PR: No nuclear reaction in the tumor cells (Allred score 0/8).

Her2: Strong complete membrane reaction in >10 % of tumor cells (score 3+)

Microscopic Examination of Left Axillary Lymph node :

Histopathological examination reveals preserved normal nodal architecture. The subcapsular
 vessels are patent. The lymph follicles are hyperactive and have prominent germinal
 centers. There is no evidence of specific granuloma or neoplasia in sections examined.

Diagnosis of Right breast cancer:

Right breast cancer, Slide revision:

Invasive duct carcinoma Grade III.

ER:

R: Positive (score 5/8).

R: Negative (score 0/8).

Verified by :

DR.Menna Nahil

Patient Name	: الأستاذة / وجودة دياب محمد عبد الرحمن	Request No.	: 13007474
Patient No.	: 27306240103145	Request Date	: 12/05/2024
Path No.	: 1535/24	Verification Date	: 19/05/2024
Sex	: Female	Financial Category	: فئة الدولة
Age	: 50 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, post NACT.

Nature of specimen: Right BCS

Gross examination:

• Procedure: Right BCS

Fibrofatty tissue piece measured 7x6.5x2 cm with covering skin ellipse measured 4.3x1.2 cm

• Tumor extent : A greyish white tumour bed site

• Size: 1x1x0.8 cm ,another rubbery grey white area 0.5x0.3x1.7 cm

• Skin involvement: Unremarkable grossly .

• Distance from margins: 1 cm away from least margin

• Lymph nodes: Fibrofatty tissue 6.5x6 cm entangling 19 nodules up to 1.5 x1 cm

• Frozen section: Free margins

Microscopic examination:

• Histological type: No residual tumor

• Ductal carcinoma in situ: No

• Skin involvement: Free

• Resection margins: Free margins

• Lymph nodes: Free (0/19)

• Histologic response to therapy: Pathologic complete response

Diagnosis:

Right BCS (post NACT) :

No residual tumor

• Free margins

• Free lymph nodes (0/19)

• Pathologic complete response (PPR)

Stage : ypT0, pN0

Dr.Rana Fathy

Verified by :

Dr. Heba Hasan

Printing Date : 25/05/2024 08:37:52

Page 1 of 2

Printing By : mervat mohamed youssef

غرفة رقم (4) - الدور الاول - مبنى رقم (287) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 287- Nile Corniche - Mizr El Qadima - Cairo, Egypt



Patient Name: Wagidah Diab Mohamed Abd El Rahman	Date : 07/04/2024
Exam : MAMMOGRAPHY BOTH	Age : 50 Year(s)
Medical ID: 4220/2023	Pat. ID: 47104

Combined bilateral breast mammography and complementary ultrasound

Indication:

right breast pathologically proven malignant mass on NACT for assessment.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- right breast shows UOQ two clips seen.
- Complementary ultrasound revealed vanishing of right breast previously seen irregular spiculated isoechoic mass lesions, was measuring 2.3 x 1.9 cm and 0.5 cm, in prior study.
- No left breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific left axillary LN.
- Free infra and supra-clavicular regions.
- Hardly appreciated previously noted right axillary level III pathological lymph nodes.

Opinion:

- Good radiological response of the right breast malignant mass lesion and axillary lymph nodes (BIRADS 6)

Yours truly

Dr.Wafaa Ali



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name : وجيدة نياز محمد عبدالحميد

Sex:

3- Medical number:
Female

83171996

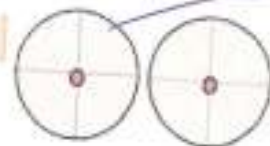
Old Med :

Old Med :

4220/2023

2-ID number: 27306240103145

Patient Age : 51 Y



Family history

Family History
-VB

Phone

Phone

01150712733

BMI

BMI

REASON OF MDT

REASON OF MDT
POST OPERATIVE

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP

Date of MDT

Date of MDT

05/06/2024

Co morbidity

Co Morbidity

dm

Menopausal
status

Menopausal Status:
POSTMENOPAUSAL

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

RT UOQ mass + multiple nodules
LT axillary LN level III & II

Mets Workup :

CT : promin carcinoma phrenic of
LT supra LNS Bone scan: free

BX
IDC

ER
5/8

PR
0/8

Her 2 Neu
3

SISH

Ki67

BX COMMENT:

ER COMMENT:

PR COMMENT:

Her 2 Neu COMMENT:

SISH COMMENT:

T
2

N
2

M

Sonomammography

PATIENT HISTORY

FOR BIOPSY FROM SCLN2ND LOOK US REVEALED NON-PATHOLOGICAL SCLN
CLIP INSERTION OF THE RIGHT BREAST MASS& SATELLITE LESION

pt received TCHP

RT WLE+ALND: no residual tumor

free surgical margins

LN 0/19

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

In view of ACHIEVING PCR YpT0N0 for :

1- PORT to rt breast and PCLN

2-ET (AI) x 5 years

3-Adj. HP (herceptin / Perjeta)) trial of one year

MEDICAL ONCOLOGY	SIGNATURE	RADIOLOGY	SIGNATURE
SURGERY	CC/17/0	PATHOLOGY	د/عبدالله
Psychiatric	د/عبدالله	Nutrition	د/عبدالله



Patient Name: Wagidah Diab Mohamed abd El		Date : 01
Exam : MAMMOGRAPHY BOTH		Age : 50
Medical ID: 4220/2023		Pat. ID: 4

Combined bilateral breast mammography and complementary ultrasound

Indication:

right breast pathologically proven malignant mass on NACT for assessment

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- right breast shows UOQ two clips seen.
- Complementary ultrasound revealed vanishing of right breast previously irregular spiculated isoechoic mass lesions, was measuring 2.3 x 1.9 cm an.
- No left breast solid or cystic masses
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific left axillary LN.
- Free infra and supra-clavicular regions.
- Hardly appreciated previously noted right axillary level III pathological lymph no

Opinion:

- Good radiological response of the right breast malignant mass lesion and axillary lymph nodes (BIRADS 6)

Yours truly

Dr. Wafaa Ali



Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mister El Qadema - Cairo, Egypt
جناح رقم (4) - الدور الأول - مبنى رقم (987) - نيل كورنيش - ميسر El Qadema - القاهرة، مصر

Printing Date : 15/02/2025 10:32:45

Page 2 of 2

Printing By : hermal

Verified by :
Dr.Heba Hasan

Dr.Heba Hassan

Pathology Report

Referring Doc :

Age

: 50 Y

Sex

: Female

Path No.

: 1535/24

Patient No.

: 27306240103145

Patient Name

: السيدة / د. هبة حسن

Registered by

: Mrs.Mayada Hegazy

Department

: Out Patient

Financial Category

: Out Patient - غير المؤمنة

Verification Date

: 19/05/2024

Request Date

: 12/05/2024

Request No.

: 13007474

Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - مختبر الأمراض النسائية



Date	Patient Name: Wagidah Diab Mohamed Abd El
Age	Rahman
Pat. ID	Exam : X-RAY CHEST
	Medical ID: 4220/2023

Plain X-ray Chest (PA view) reve

- Clear both lung fields.
- Clear both costo-phrenic angles.
- Normal cardiac size & configuration.
- Normal thoracic cage

Signature Dr. Nem
22/01/2



Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department



وزارة الصحة والسكان
المركز الطبي التخصصي
دار السلام للسرطان
قسم الأشعة
التاريخ: 2025/01/22

Patient Name: Wagidah Diab Mohamed Abd El Rahman	Medical ID: 4220/2023
Exam : ultrasound ABDOMEN& PELVIS	Age : 51 Year(s)
Date : 22/01/2025	Pat ID: 47104

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- enlarged in size (18cm at Rt MCL) bright parenchymal echopattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilation.

Gall Bladder: -

- Portal vein is patent and not dilated.
- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen: -

- Average size, homogenous.

Kidneys: -

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross

abnormalities.

No ascites

Urinary Bladder: -

- Average capacity with no stones or diverticular out pouching

Uterus and adnexal: no abnormalities could be detected

Opinion:

Bright hepatomegaly likely fatty
Colonic gaseous distention

Signature Dr . Nermien Ramadan
22/01/2025 3:19 PM



Patient Name: Wagidah Diab Mohamed Abd El Rahman	Date : 15/02/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 51 Year(s)
Medical ID: 4220/2023	Pat. ID: 47104

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had CBS in

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- ACR:A.
- No left breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Right breast focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast focal parenchyma distortion, owing to prior surgical interference with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative benign sequel, (BIRADS 3) for follow up after 6 months
- Normal study of the left breast (BIRADS 1)

Yours truly

Signature Dr. Youmna tarek

15/02/2025 2:04 PM



Echo report

Name: د. محمد د. د. د.

Date: 2/11/2024

229
2

EF	60%	55-75%	PWT	41	0.6-1.1cm
LVEDD	5.7	3.5-5.8cm	AO.R	3.6	2-3.7cm
LVESD	3.9	2-4cm	LA	3.9	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT		0.6-1.1cm	AO.CUSP		1.5-2.5cm

LV Dimensions (Normal)

LV wall thickness (Normal)

LV diastolic function (Normal)

SWMA (Normal)

LA and Aortic root dimensions (Normal)

Mitral Valve (Normal)

Aortic valve (Normal)

Tricuspid valve (Normal)

Pulmonary valve (Normal)

Right side and systolic function (Normal)

Intracardiac masses and effusion (Normal)

Conclusion: Normal Echo cardigrapy study

Msc cardiology

د. محمد د. د. د.



Verified by :
Dr.Heba Hasan

Dr.Rana Fathy

- Stage : ypT0, ypN0
- Pathologic complete response (PPR)
- Free lymph nodes (0/19)
- Free margins
- No residual tumor
- Right BCS (post NACT) :
- Histologic response to therapy: Pathologic complete response
- Lymph nodes: Free (0/19)
- Resection margins: Free margins
- Skin involvement: Free
- Ductal carcinoma in situ: No
- Histological type: No residual tumor
- Microscopic examination:
- Frozen section: Free margins
- Lymph nodes: Fibrofatty tissue 6.5x6 cm entangling 19 nodules up to 1.5 x1 cm
- Distance from margins: 1 cm away from least margin
- Skin involvement: Unremarkable grossly.
- Size: 1x1x0.8 cm ,another rubbery grey white area 0.5x0.3x1.7 cm
- Tumor extent : A greyish white tumour bed site
- Fibrofatty tissue piece measured 7x6.5x2 cm with covering skin ellipse measured 4.8x1.2 cm
- Procedure: Right BCS
- Gross examination:
- Nature of specimen: Right BCS
- Clinical Data: Right breast cancer, post NACT .

Pathology Report

Referred by	Department	Registered by	Referring Doc :
Request No.	Request Date	Verification Date	Path No.
13007474	12/05/2024	19/05/2024	1535/24
27306240103145	12/05/2024	19/05/2024	1535/24
Female	Category	Out Patient	Age
50 Y	Financial Category	Out Patient	Sex
50 Y	Financial Category	Out Patient	Age
50 Y	Financial Category	Out Patient	Age





Patient Name	: الأستاذ / وجيهه نجيب محمد عبدالرحمن	Request No.	: 13007474
Patient No.	: 27306240103145	Request Date	: 12/05/2024
No.	: 1535/24	Verification Date	: 19/05/2024
Sex	: Female	Financial Category	: اورام ندر السلام - لفقة قبول
Age	: 50 Y	Department	: Out Patient
Referring Doc		Registered by	: Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, post NACT .

Nature of specimen: Right BCS

Gross examination:

- Procedure: Right BCS

Fibrofatty tissue piece measured 7x6.5x2 cm with covering skin ellipse measured 4.8x1.2 cm

- Tumor extent : A greyish white tumour bed site
- Size: 1x1x0.8 cm .another rubbery grey white area 0.5x0.3x1.7 cm
- Skin involvement: Unremarkable grossly .
- Distance from margins: 1 cm away from least margin
- Lymph nodes: Fibrofatty tissue 6.5x6 cm entangling 19 nodules up to 1.5 x1 cm

• Frozen section: Free margins

Microscopic examination:

- Histological type: No residual tumor
- Ductal carcinoma in situ: No
- Skin involvement: Free
- Resection margins: Free margins
- Lymph nodes: Free (0/19)
- Histologic response to therapy: Pathologic complete response

Diagnosis:

Right BCS (post NACT) :

No residual tumor

- Free margins
- Free lymph nodes (0/19)
- Pathologic complete response (PPR)
- Stage : ypT0, ypN0



ERM036724575

Dr.Rana Fathy

Verified by :

Dr.Heba Hasan

Printing Date : 15/07/2025 10:32:45

Page 1 of 2

Printing By : hermal

1- Right breast,TRU-cut biopsy:

Invasive duct carcinoma,grade III

2- Left axillary Lymph node,TRU-cut biopsy:

Reactive nodal tissue.Negative for metastasis in examined cores. == For clinical & radiological correlation.

Comments

ER, PR & HER-2 are ordered.

Co-signed by:

Signature

MAHDIAB IBRAHIM MOHAMMED MD

Reported and electronically signed by
Wahab Mohamed Elshamy Mohamed

Validated on 23/10/2023 02:44 PM

Print Date: 07/11/2023 10:14 AM

Page number: 1 of 2

MRN:	10123232982
Patient Name:	WAGDAD DAYAB MOHAMMED ABD ELRAHMAN
DOB:	24/08/1973
ID No.:	27306240113145
Sex:	Female
Location:	OncoMed Clinic

IMMUNOPATHOLOGY

Her2 Result

Specimen Id	2310280211	Collection Date	20/10/2023 12:28 PM
Accession No	47/123-5308	Received Date	20/10/2023 12:28 PM
Sample Type	Paraffin Block	Doctor	Abd ALI AH Mohamed Gamil

History, Relevant Clinical Findings & Previous Inv. Results:	Reason for Order:
--	-------------------

Immunohistochemical Report

pathology number

7911/23

Clinical Data

RT breast cancer

Gross Picture :

Referred block submitted for Her2 Immunostaining.

System Used :

1.

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-Her2-2 [4B5] rabbit monoclonal antibody.

Scoring system :

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2018. IHC 0 is defined by no

staining observed or membranous staining that is incomplete and is faintly/barely perceptible and

within <=10% of the invasive tumor cells. IHC 1+ is defined by incomplete membrane

staining that is faintly/barely perceptible and within >10% of the invasive tumor cells. IHC 2+ :

The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining

observed in >10% of tumor cells. IHC 3+ : based on circumferential membrane staining that is

complete, intense in >10 % of tumor cells

Laterality - 1

Right

Her2 Result - 1

Positive [score 3+]

Signature

MAHMOUD IBRAHIM MOHAMED PHD

Co-Signed by:

Reported and electronically endorsed by
Mahmoud Mohamed Elshewy Mohamed

MRN:	10173233PRD
Patient Name:	WAZIADA OATYAB NOHAMMED ABD ELRAHMAN
D.O.B:	24/06/1973
ID No.:	PT200340103145
Sex:	Female
Location:	External Clinic

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id	2310290208	Collection Date	28/10/2023 11:59 AM
Accession No	14425-8307	Received Date	28/10/2023 12:29 PM
Sample Type	Paraffin Block	Doctor	Abd-ALLAH Mohamed Gamil

History, Relevant Clinical Findings & Previous IHC Results:

Reason for Order:

Immunohistochemical Report

pathology number

7914/23

Clinical Data:

RT breast cancer.

Gross Picture :

Referred block submitted for ER/PR immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-PR [LE2] Rabbit monoclonal antibody, Ventana anti-ER[SPL] Rabbit monoclonal antibody.

Scoring system :

According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample is reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allied score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

Comment:

- There are much more limited data on the 2% to 3% of cancers that are low ER expressors (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.
 - PR positive in 1-20% of tumor cells is considered as Low positive.
- (Laterality) Breast Cancer

Right

Diagnosis :

RT breast cancer

ER : Low positive, strong nuclear reaction in 5% of tumor cells (5/8).

PR :Negative, (0/8).

Signature

HAYITAD IBRAHIM MOHAMMED MD



Women's Health Outreach Program

Name: wagida diab mohmed

Date: 17/12/2024

Ultrasound guided clip insertion

Left breast

- Under complete aseptic condition & ultrasound guidance two clip insertion was done for right breast two malignant mass lesions at 9 o'clock para areolar and deep zone b the clips were inserted at the centre of both lesion and this confirmed by mammographic CC & ML

Best Hegards,

Dr. Marwa M.Hassan

WOMEN IMAGING DEPARTMENT

Patient Name: Mrs/ Wageda Diab
Department:

Ref.
Date: 11-5-2024

Clinical background: Reported case of right breast multifocal carcinoma upper outer with inserted clips, post chemotherapy and complete disappearance of the masses. Clips was appreciated at mammogram with intervening distance of 3 cm on mammogram for wire localization.

Mammogram guided wire insertion:

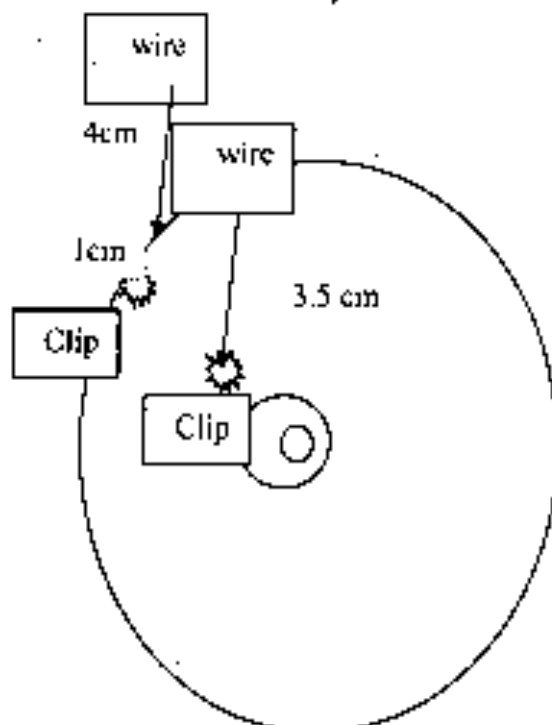
Two clips was appreciated upper outer targeted under aseptic condition.

Anterior clip: The wire tip was inserted perpendicular for 3.5 cm. targeting the anterior clip exactly on it.

Posterior clip: The wire tip was inserted perpendicular for 5 cm. targeting the posterior clip within the sheath, yet at the time of sheath removal and imaging the wire tip was lifted for 1 cm distance upward away from the clip (i.e. the wire length within the breast is 4 cm and the clip is 1 cm inferior to the wire tip within the breast)

Confirmation by CC and ML 90 views was performed.

Much obliged
Sahar Mansour



PATIENT NAME: WAGADA DAYAB MOHAMMED ABD EI RAHMAN

GENDER: Female

DATE : 24-Oct-2023 09:13 AM

PATIENT ID: 10723233983

AGE: 50 y

ORDER: 2546409.2546410

Clinical indication - all studies

Report: - all studies

FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

- Scattered fibroglandular densities (ACR b).
- No grouped microcalcifications. Bilateral few scattered punctate calcification and vascular atheromatous calcification.
- **The right breast showed upper outer indistinct mass that measured 2.3 x 1.9 x 4 cm. In a distance of 2 cm another minion is noted (0.5 cm) . The overall area of extension measured 5.24 cm length.**
- No left breast obvious masses could be detected.
- No lactiefrous duct dilaiation.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- The right axilla showed matted nodes, yet average sized, with preserved fatty hilum and uniform cortical thickening of 0.3 cm.
- **The left axilla showed few level III pathological nodes of globular appearance indistinct fatty hilum and the largest measured 1.2 x 0.8 cm; cortical thickening of 0.5 cm. Level II presented multiple indeterminate nodes of asymmetrical cortical thickening of 0.4 cm yet with preserved fatty hilum.**
- No supraclavicular, infraclavicular or inetrnal mammary nodes.
- N.B. Right breast upper inner 2 clock dermal mole that measured 1.3 x 0.3 cm

OPINION:

Right breast upper outer (9 clock) malignant looking solid mass (BI-RADS 5)

PATIENT: WAGADA DAYAB MOHAMMED ABD EI RAHMAN, 10723233983, Female, 50 y

Report version 2, 27-Nov-2023 09:25 AM

Page 1 of 2



Ministry of Health, Egypt



Ministry of Health, Egypt

Ministry of Health, Egypt

Mammogram
[DOCTYPE:MAMMOGRAM]

that showed multifocal distribution, for biopsy confirmation. Concomitant
contra-lateral (left) axillary level III small sized few small sized pathological
nodes and bilateral level II indeterminate likely reactionary nodes.
Bilateral scattered benign and vascular calcification.

Recommendation - all studies

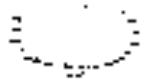
Referral to clinic

Reported By,

Prof.Dr. SAHAR MANSOUR

24-Oct-2023 12:54 PM





Patient Name	: الأستاذة / وجيدة فهاب محمد عبدالرحمن	Request No.	: 13007474
Patient No.	: 27306240103145	Request Date	: 12/05/2024
Path No.	: 1535/24	Verification Date	: 19/05/2024
Sex	: Female	Financial Category	: اورنم دار السلام - نفقة المتولة
Age	: 50 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, post NACT .

Nature of specimen: Right BCS

Gross examination:

- Procedure: Right BCS

Fibrofatty tissue piece measured 7x6.5x2 cm with covering skin ellipse measured 4.8x1.2 cm

- Tumor extent : A greyish white tumour bed site
- Size: 1x1x0.8 cm ,another rubbery grey white area 0.5x0.3x1.7 cm
- Skin involvement: Unremarkable grossly .
- Distance from margins: 1 cm away from least margin
- Lymph nodes: Fibrofatty tissue 6.5x6 cm entangling 19 nodules up to 1.5 x1 cm
- Frozen section: Free margins

Microscopic examination:

- Histological type: No residual tumor
- Ductal carcinoma in situ: No
- Skin involvement: Free
- Resection margins: Free margins
- Lymph nodes: Free (0/19)
- Histologic response to therapy: Pathologic complete response

Diagnosis:

Right BCS (post NACT) :

No residual tumor

- Free margins
- Free lymph nodes (0/19)
- Pathologic complete response (PPR)
- Stage : ypT0, ypN0

Dr.Rana Fathy

Verified by :

Dr.Heba Hasan

Printing Date : 25/05/2024 08:37:52

Page 1 of 2

Printing By : ,marwa mohamed youes



غرفة رقم (4) - الدور الأرضي - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First floor-Building No. 987-Nile Corniche - Mizr El Qalima - Cairo, Egypt

PATIENT NAME: WAGADA DAYAB MOHAMMED ABD ELRAHMAN

GENDER: Female

DATE : 05-Nov-2023 10:03 AM

PATIENT ID: 10123233983

AGE: 50 y

ORDER: 2558021

Report:**99mTc-MDP BONE SCAN****Desired Information:** -
skeletal survey.**Technique:**

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

No hot spot that account for osseous deposits could be detected all over the surveyed skeleton.

Degenerative changes are seen over dorso-lumbar vertebrae, more at L4/5 & L5/S1.

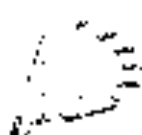
Arthritic changes are seen over both knees.

Both kidneys are normally visualized.

Conclusion:

No definite evidence of metastatic osseous deposits.

Reported By,**Ass. Prof. Huda Fathy Elsayed****06-Nov-2023 05:54 PM**



Personal Progress for Women's Health
Ministry of Health and Population

البرنامج الشخصي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name: رجدة ليلي محمد عبدالحسين

Sex:

3- Medical number:

Female

83171576

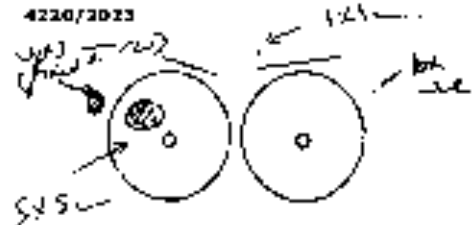
Old Med:

Date:

4/22/2023

2-ID number: 27306240103145

Patient Age: 50 Y



Family history

1st & 2nd
-ve F.M

Phone

Pass

0150717732

BMI

Ref

31

REASON OF MDT

CLINICAL & RPT

marital status

Married

MRS Score

MO score

1

G&PJP

CT/PT

1

Date of MDT

MDT ID NO

13/12/2023

Co-morbidity

No history
DM: none

Menopausal
status

Menopausal Status
NOTHROPASAL

Initial STAGE

1st STAGE

Clinical data

Breast Imaging:

RT UOQ mass + multiple nodules
LT axillary LN level I-II & III

Mets Workup:

CT: perimen carcinoma phrenic of
LT supra LNS Bone scan: Free

BX

ER

S/B

PR

0/R

Her 2 Neu

3

SIH

K107

SLIDE NO

RT mass IDC GIII

LT axillary LN react I-II

PR COMMENT

PR COMMENT

HER 2 COMMENT

SIH COMMENT

T

2

N

2

M

Sonamammography

PATIENT HISTORY

FOR BIOPSY FROM SCLN

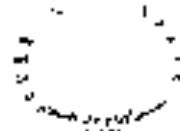
CLIP INSERTION OF THE RIGHT BREAST MASS& SATELLITE LESION

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

FOR TCHP FOR 6 CYCLES GUIDED BY CLINICAL REPONSE THEN CONSIDER RT WLE& ALND

SIGNATURE			
MEDICAL ONCOLOGY	MDT OUTCOME & PLAN	RADIOLOGY	MDT OUTCOME & PLAN
SURGERY	MDT OUTCOME & PLAN	PATHOLOGY	MDT OUTCOME & PLAN
Psychiatric	MDT OUTCOME & PLAN	Nutrition	MDT OUTCOME & PLAN



Date : 22/02/2024	Patient Name: Wagidah Diab Mohamed
Age : 50 Year(s)	Exam : ultrasound ABDOMEN & PELVIS
	Medical ID: 4220/2023

**ABDOMINO-PELVIC ULTRASOUND EXAMINATION
REVEALED:**

Liver: -

- Enlarged in size (about 19.8 cm at the right MCL), smooth outline, bright echopattern. No intra hepatic biliary radicle dilatation.
- Portal vein is not dilated and patent.

Gall Bladder: -

- Average size, normal wall thickness, no stones
- CBD is not dilated.

Spleen: -

- Border line (About 13.2 cm), displaying homogenous echopattern.

Kidneys: -

- Are of normal size and site, normal echogenicity, no stone or backpressure changes, no solid or cystic lesion. Left renal gravel.
- Para-aortic and pancreatic area: no gross lesion

URINARY BLADDER: is empty during examination

Marked colonic and gaseous distension

No free fluid in the Douglas pouch.

Opinion:

Bright hepatomegaly.

Border line spleen

Marked colonic and gaseous distension

Best Regards,
Dr. Dina Gatter FRCR,

Not Approved



Echo report

MR ٤٤٤
٢٢

Name:

وحيد محمد ريان

Date: 2/5/2024

EF	63%	55-75%	PWT		0.6-1.1cm
LVEDD	4.9	3.5-5.8cm	AD.R	3.4	2-3.7cm
LVESD	3.1	2-4cm	LA	3.3	2-4cm
FS	34%	25-45%	RV		1.7-2.3cm(supine)
SWT	1.0	0.6-1.1cm	AQ.CUSP		1.5-2.6cm

LV dimensions

LV wall thickness

LV diastolic function DD GI

SWMA No SWMA at rest, IHD couldn't be excluded.

LA and Aortic root dimensions

Mitral Valve

Aortic valve

Tricuspid valve

Pulmonary valve

Right side and systolic function

Intracardiac masses and effusion

D/Mohamed Ali

Msc cardiology



Echo report

وحيده دينا

MR: 440
12

Name:

Date: 3/4/2024

EF	66	55-75%	PWT	11	
LVEDD	412	3.5-5.8cm	AO.R	217	2-3.7cm
LVEDD	219	2-4cm	LA	318	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT		0.6-1.1cm	AD.CUSP		1.5-2.6cm

LVDimensions Concentric LVA

LV wall thickness normal

LV diastolic function mild CM

SWMA normal

LA and Aortic root dimensions normal

Mitral Valve mild MR

Aortic valve normal

Tricuspid valve normal

Pulmonary valve normal

Right side and systolic function normal

Intracardiac masses and effusion no masses, no effusion

DMsc cardiology

[Signature]

PATIENT NAME: WAGADA DAYAB MOHAMMED ABD ELRAHMAN

GENDER: Female

DATE : 31-Oct-2023 03:14 PM

PATIENT ID: 10123233683

AGE: 50 y

ORDER: 2558019,2558020

Report: - all studies

Multislice post contrast CT scan of the chest, abdomen and pelvis revealed:

(I) Chest:

- The right breast shows an enhancing lesion in the LOQ measuring about 25 x 22 mm in its maximum axial dimensions, associated with mild edematous changes and skin thickening.
- Bilateral prominent axillary lymph nodes are seen.
- A prominent left supraclavicular lymph node is seen measuring about 8 x 5 mm in its maximum dimensions.
- Patent central air ways with well inflated both lungs.
- Both lung fields are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- A prominent right cardiophrenic lymph node is seen measuring 9 x 6 mm in its maximum dimensions.
- Otherwise, No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is markedly enlarged in size showing homogenous low CT density and enhancement with no focal lesions.
- Patent and homogeneously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- Left iliac antero-lateral abdominal wall hernia with fat content
- No retro-crural, para-aortic or pelvic lymphadenopathy.

PATIENT: WAGADA DAYAB MOHAMMED ABD ELRAHMAN, 10123233683, Female, 50 y

Report version 2, 07-Nov-2023 09:21 AM

Page 1 of 2



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16602

مركز بحية البحر
و جوارها في جوار البحر و جوارها في جوارها

مستشفى بحية البحر
في جوارها في جوارها في جوارها

[DOCTYPECT_SCAN]

- No ascites.
- IUD is noted
- The examined axial and appendicular skeleton show mild spondylodegenerative changes with no significant osseous lesions

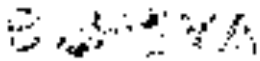
OPINION:

- **Right breast neoplastic process**
- **Prominent right cardiophrenic and left supraclavicular lymph nodes**
- **No CT evidence of definite pulmonary , hepatic or osseous metastatic deposits**

Reported By,
Dr. Ahmed Abdelmoneim, MSc
Dr. Mennatallah Mohamed, MD

01-Nov-2023 06:12 PM





Ministry of Health, Egypt

مركز رعاية الهرم
مركز طبعة اول عازن الهرم بجوار طريق الجيزة
تم تأميم المستشفى

PATIENT NAME: WAGADA DAYAB MOHAMMED ABD ELRAHMAN
GENDER: Female
DATE : 05-Nov-2023 10:03 AM

PATIENT ID: 10123233963
AGE: 50 y
ORDER: 7558021

Report:

99MTC-MDP BONE SCAN

Desired Information: -
Skeletal survey.

Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 25 mCi 99m Tc-MDP in the following manner:

Report:
Hot spot that account for osseous deposits could be detected all over the surveyed skeleton.
Degenerative changes are seen over dorso-lumbar vertebrae, more at LV2&3
Arthritic changes are seen over both knees
Both kidneys are normally visualized.

Conclusion:
Definite evidence of metastatic osseous deposits.

Reported By,
Dr. Prof. Huda Fathy Elsayed

06-Nov-2023 05:54 PM

WT WAGADA DAYAB MOHAMMED ABD ELRAHMAN, 10123233963, Female, 50 y
Version 1, 07-Nov-2023 09:06 AM

www.Bahaya.org
 16602

مركز رعاية الهرم
مركز طبعة اول عازن الهرم بجوار طريق الجيزة

مستشفى رعاية الشيخ زايد
6 احياء الطول - الشيخ زايد - اقليم كالة الجديدة



Patient Name	: الأمثلة / وجيدة نيل محمد عبدالرحمن	Request No.	: 13001729
Patient No.	: 27306240103145	Request Date	: 09/11/2023
Path No.	: SR-1361/23	Verification Date	: 26/11/2023
Sex	: Female	Financial Category	: اوزاع دار السلام - نفقة الادارة
Age	: 50 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer and Left axillary lymph node.

Nature of Specimen: Slide Revision

Gross Examination:

- 2 paraffin block(s) labeled by (7914/23) and (3) slides were received.

Microscopic Examination of Right breast:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitoses) Grade III (3+3+2)
- * DCIS: No.
- * Angiolinvasion: No.
- * Perineural invasion: No.
- * IPT:
- ER: Strong nuclear reaction in 10% of tumor cells (Allred score 5/8).
- PR: No nuclear reaction in the tumor cells (Allred score 0/8).
- Her2: Strong complete membrane reaction in >10 % of tumor cells (score 3+).

Microscopic Examination of Left Axillary Lymph node :

histopathological examination reveals preserved normal nodal architecture. The subcapsular sinuses are patent. The lymph follicles are hyperactive and have prominent germinal centers. There is no evidence of specific granuloma or neoplasia in sections examined.

Diagnosis of Right breast cancer:

Right breast cancer, Slide revision;

Invasive duct carcinoma Grade III.

IPT:

- ER: Positive (score 5/8).
- PR: Negative (score 0/8).

Verified by :

DR.Menna Nabil





مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

MDT FORM

MDT FORM

115/2024-GA

Name	وسيلة دياب مكي	MRN	4220/2023
Age	50	ID	AMIV/1997
Menopausal status	postmenopausal	Phone	11-59-VIPV
NRS SCORE	1	Marital status	11-58-1-2V
BMI	30	Family history	-ve
CO morbidity	DM	REASON OF MDT	P.C

Clinical Data

BX: IDC G3

ER: 5/8

PR: 2/8

HER2: +3

Ki67:

Sonomammography:

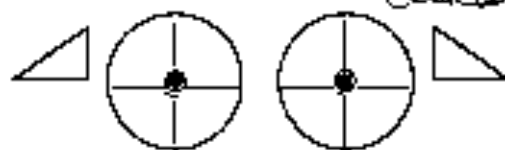
Mets Workup: CT2N2 ~~metastatic~~

Bone scan
is free
CT LAP is prominent
right cardiophrenic, ct scan

R

PR: NAC

clinical R



TNM

Recommendations:

with good clinical response

Breast

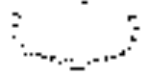
for MDT

MDT OUTCOME & PLAN

For core evaluation of both breasts the R. LAP & MDT

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
P. G. / C. C. / 0/1	[Signature]	[Signature]		[Signature]	



Patient Name	: الأستاذة / وجيدة تيارب محمد عبدالرحمن	Request No.	: 13007474
Patient No.	: 27306240103145	Request Date	: 12/05/2024
Path No.	: 1535/24	Verification Date	: 19/05/2024
Sex	: Female	Financial Category	: اورام دار السلام - غفلة الشولة
Age	: 50 Y	Department	: Out Patient
Referring Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, post NACT .

Nature of specimen: Right BCS

Gross examination:

• Procedure: Right BCS

Fibrofatty tissue piece measured 7x6.5x2 cm with covering skin ellipse measured 4.8x1.2 cm

• Tumor extent : A greyish white tumour bed site

• Size: 1x1x0.8 cm .another rubbery grey white area 0.5x3.3x1.7 cm

• Skin involvement: Unremarkable grossly .

• Distance from margins: 1 cm away from least margin

• Lymph nodes: Fibrofatty tissue 6.5x6 cm entangling 19 nodules up to 1.5 x1 cm

• Frozen section: Free margins

Microscopic examination:

• Histological type: No residual tumor

• Ductal carcinoma in situ: No

• Skin involvement: Free

• Resection margins: Free margins

• Lymph nodes: Free (0/19)

• Histologic response to therapy: Pathologic complete response

Diagnosis:

Right BCS (post NACT) :

No residual tumor

• Free margins

• Free lymph nodes (0/19)

• Pathologic complete response (PPR)

• Stage : ypT0, ypN0

Dr.Rana Fathy

Verified by :

Dr.Haba Hasan

Printing Date : 25/05/2024 08:37:52

Page 1 of 2

Printing By : marwa mohamed youss



هاتف رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Nasr El Qudous - Cairo, Egypt



Ministry of Health and Population
Ministry of Health and Population

الوزارة العامة للصحة والسكان
وزارة الصحة والسكان



MDT form

Applicant ID:

1- Name : وجدة بواب محمد عبدالحسن

Sex:

3- Medical number:

Female

RD 171946

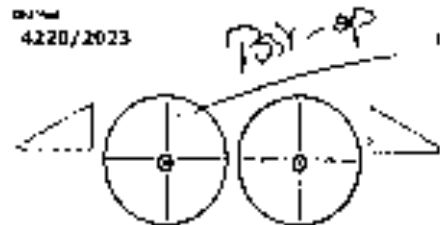
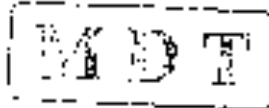
Old Regd .

Date:

4/220/2023

2-ID number: 27306240103145

Patient Age : 51 Y



Family history	Family history No	Phone	Phone 01150712731	BMI	BMI 24	Reason of MDT	Reason of MDT MDT referral per
Marital status	Marital status Married	BRS Score	BRS Score No score	CRCA	CRCA A	Date of MDT	Date of MDT 05/06/2024 M
Co-morbidity	Co-morbidity No	Menopausal status	Menopausal status No	Local stage	Local stage No		

Clinical data

Breast Imaging :

RT UOQ mass + multiple nodules
LT axillary L4 level III & II

Mets Workup :

CT : promin carcinoma phrenic of
LT supra LNS Bone scan: free

BX EDC BX COMMENT	ER 5/8 ER COMMENT	PR Q/B PR COMMENT	Her 2 Neu 3 Her 2 Neu COMMENT	SIEN SIEN COMMENT	Ki67
-------------------------	-------------------------	-------------------------	-------------------------------------	----------------------	------

T 2	N 2	M
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Sonomammography

PATIENT HISTORY

FOR BIOPSY FROM SCLN2ND LOOK US REVEALED NON-PATHOLOGICAL SCLN
CLIP INSERTION OF THE RIGHT BREAST MASS& SATELLITE LESION

pt received TCHP

RT WLE+ALND: no residual tumor

free surgical margins

LN 0/19

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

in view of ACHIEVING PCR YpT0N0 for :

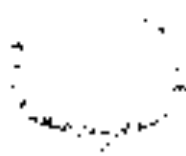
1- PORT to rt breast and PCLN

2-ET (AI) x 5 years

3-ADJ. HP (herceptin / Perjeta) } trial of one year

SIGNATURE

MEDICAL ONCOLOGY	C. C. 17/10 Dr. Mohamed Elmaghrabi	RADIOLOGY	Dr. Mohamed Elmaghrabi
SURGERY	Dr. Mohamed Elmaghrabi	PATHOLOGY	Dr. Mohamed Elmaghrabi
Psychiatric	Dr. Mohamed Elmaghrabi	Nutrition	Dr. Mohamed Elmaghrabi



Patient Name: Wagidah Diab Mohamed Abd El Rahman	Date : 15/02/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 51 Year(s)
Medical ID: 4220/2023	Pat. ID: 47104

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patrent had CBS in

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- ACR:A.
- No left breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Right breast focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast focal parenchyma distortion, owing to prior surgical interference with with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative benign sequel, (BIRADS 3) for follow up after 6 months
- Normal study of the left breast (BIRADS 1)

Yours truly

Signature Dr. Youmna tarek

15/02/2025 2:04 PM



Echo report

Name: *د. عبد الله*

Date: 2/11/2024

258
2

EF	60%	55-75%	PWT	4	0.6-1.1cm
LVEDD	5.7	3.5-5.8cm	AO.R	3.6	2-3.7cm
LVEDS	3.9	2-4cm	LA	3.9	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT		0.6-1.1cm	AO.CUSP		1.5-2.3cm

LV Dimensions (Normal)

LV wall thickness (Normal)

LV diastolic function (Normal)

SWMA (Normal)

LA and Aortic root dimensions (Normal)

Mitral Valve (Normal)

Aortic valve (Normal)

Tricuspid valve (Normal)

Pulmonary valve (Normal)

Right side and systolic function (Normal)

Intracardiac masses and effusion (Normal)

Conclusion: Normal Echo cardiography study

Msc cardiology

د. عبد الله

Patient Name: Wagideh Diab Mohamed Abd El Rahman	Date : 16/08/2025
Exam : Ultrasound BREAST (BOTH SIDE)	Age : 32 Year(s)
Medical ID: 4720/2023	Pat. ID: 47104

BILATERAL BREAST ULTRASONOGRAPHY REVEALED:

Indication: Follow up after right conservative surgery.

- Homogenous (predominantly faty) parenchymal echotexture.
- Thickened right breast skin (4 mm) with underlying minimal interstitial edema.
- No detected suspicious solid or cystic masses in both breasts..
- No detected significant axillary lymph nodes.

CONCLUSION:

- Post conservative surgery changes of the right breast (**BIRADS 2 Category**).
- Normal left breast (**BIRADS 1 Category**).

With Regard

DR. MONA ZAKY.MD

Professor of diagnostic radiology-Breast Imaging unit -Mansoura University

Signature Dr. Mona Zaki

15/08/2025 12:36 PM



Patient Name: Wagidah Diab Mohamed Abd El Rahman	Date : 15/02/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 51 Year(s)
Medical ID: 4220/2023	Pat. ID: 47104

**BI LATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had CBS in

Technique

- Bilateral MLO and CC views
- Bi lateral breast ultrasound

Report:

- ACR:A.
- No left breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Right breast focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast focal parenchyma distortion, owing to prior surgical interference with with no evidence of recurrent mass lesions.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative benign sequel, (BIRADS 3) for follow up after 6 months
- Normal study of the left breast (BIRADS 1)

Yours Truly

Signature Dr Youmna tarek

15/02/2025 2:04 PM



Patient Name: Wagidah Diab Mohamed Abd El Rahman	Date : 22/01/2025
Exam : X-RAY CHEST	Age : 51 Year(s)
Medical ID: 4220/2023	Pat. ID: 47104

Plain X-ray **Chest** (PA view) revealed:

- Clear both lung fields.
- Clear both costo-phrenic angles.
- Normal cardiac size & configuration.
- Normal thoracic cage

Signature Dr . Nermien Ramadan

22/01/2025 3:16 PM